AURELIA · FEEDBACK GROUP FG-2026-01 · PROJECT MANAGER PLAYBOOK · 11 NOVEMBER 2026

Eight weeks to find out whether Aurelia gets people better

For the project manager who will recruit up to 30 people with different wellness problems and guide them every day, using Aurelia, from the problem to the healthy range. What each milestone is for, what you do, how you know it is passed, and where the admin shows it.



A win: “bad sleep → good sleep” means moving into the healthy band of their own questionnaire, not only a better score. Illustration, not data.

30 people · six issues | 8 weeks + 3 to prepare | Source of truth: admin → Experimentation

AI analysis · Based on Claude self-analysis and deep research

1. Why this matters

This is the product-market-fit test for Aurelia. Everything built so far rests on one promise: that a person with a common wellness problem can describe it in a sentence and get better with sessions made for them. Until real people with real problems get better, that promise is a design. In eight weeks we will know whether it holds, for whom, and what has to change.

People solve their issue

They move from the problem range into the healthy range of their own questionnaire. That is a win, and it is what the study is for.

They keep using it

Three or more sessions a week without being pushed harder each week. An effect without use is noise; use without effect is a finding.

They would miss it

At least 40% would be “very disappointed” to lose Aurelia: the usual product-market-fit bar.

The hypothesis. People with a common wellness problem who use Aurelia three or more times a week, with light daily guidance, move from the problem range into the healthy range of their own issue within eight weeks, and would be very disappointed to lose it. **What would disprove it:** people do not use it even with your guidance, or they use it and do not change, or they change and would not miss it.

2. Your role

You are the guide, Aurelia does the work

Your job is to keep people playing, listening and honest about how they feel. The sessions, the chapters and the changes come from Aurelia. When you are tempted to give advice, ask a question instead and pass what you heard to the product team.

You are the study's memory

Every contact goes in the coaching log the same day. If it is not in the admin, it did not happen, and nobody can learn from it.

You are not a clinician

You never interpret a score to a participant, never suggest stopping a treatment, and never handle a crisis alone. Safety goes to the study lead the same day.

3. Six issues, and what a win means for each

Each person joins one cohort: the goal they choose in Aurelia's onboarding, and one validated questionnaire for it. Recruit to these numbers.

COHORT	APP GOAL	THEIR QUESTIONNAIRE	HEALTHY BAND (A WIN)	MEANINGFUL CHANGE	PEOPLE
Bad sleep → good sleep	Sleep better	Insomnia Severity Index (ISI), 0–28	7 or less: no insomnia	6 points lower	6
Anxious → calm	Feel calmer	GAD-7, 0–21	4 or less: minimal	4 points lower	6

COHORT	APP GOAL	THEIR QUESTIONNAIRE	HEALTHY BAND (A WIN)	MEANINGFUL CHANGE	PEOPLE
Mental fog → clarity	Clear my mind	PROMIS Cognitive Function (T-score)	45 or more: within normal	5 points higher	5
Low energy → energy	More energy	PROMIS Fatigue (T-score)	54 or less: within normal	5 points lower	5
Distracted → focused	Focus	PROMIS Cognitive Function (T-score)	45 or more: within normal	5 points higher	4
Stressed body → regulated	Regulate my body	Perceived Stress Scale (PSS-10), 0–40	13 or less: low stress	4 points lower	4

Everyone also answers WHO-5 (well-being, 0–100) every two weeks: a gain of 10 points is meaningful, and 28 or below is a cue for a person to follow up. A clinical advisor confirms the thresholds before week 0.

4. The eleven weeks at a glance



Questionnaires: WHO-5 at weeks 0, 2, 4, 6 and 8; the cohort measure and PHQ-9 at weeks 0, 4 and 8; interviews at weeks 0, 4 and 8. A check-in after every session.

5. The six milestones

Each milestone has a gate: the lines that say it is passed. The admin computes the same gates in **Version 2 → Milestones**, so you never have to work them out by hand.

M0 Set up and recruit

Three weeks before week 0

Thirty people across six issues, consented and screened, with the study tools ready.

YOU

- Recruit 4–6 people per issue (table in section 3), from outside the team and outside anyone's friends.
- Screen with PHQ-9 and the exclusion list; anyone excluded is offered support resources, not dropped silently.
- Take informed consent; give each person a pseudonymous id (P01–P30) and keep the name key outside every tool.
- Book every baseline call and agree each person's daily time (for sleep, the evening).

THE PRODUCT TEAM

- Accounts that stay signed in, study enrolment, the questionnaire screens and the safety alert (protocol section 10).
- A clinical advisor and an ethics review confirm the measures and the safety process.

GATE: PASSED WHEN

☐ 30 enrolled, every cohort filled

☐ Ethics review and clinical advisor: confirmed by a person

If it is not: Do not start with fewer than 24. Extend recruitment a week rather than start a cohort with one person.

See it in admin →

M1 Baseline and first session

Week 0

Everyone measured before they start, and playing within two days.

YOU

- Collect WHO-5, the cohort measure and PHQ-9 before the first session. A baseline after use is not a baseline.
- Interview 1 (30 minutes): what the problem is like, what they have tried, what "solved" would mean to them, in their words.
- Sit with each person (call or in person) for their first session: onboarding, the first sentence, the first play.
- Write their goal sentence in the coaching log: it is what you will measure the next eight weeks against.

THE PRODUCT TEAM

- Watches the first sessions for blockers and fixes the ones that stop people playing.

GATE: PASSED WHEN

☐ Baseline WHO-5 and cohort measure: 100%

☐ Played a first session in week 1: 90% or more

If it is not: Anyone without a baseline does not count for outcomes. Collect it before they play again.

See it in admin →

M2 Build the habit

Weeks 1–2

A daily rhythm: three or more sessions a week, a check-in after each.

YOU

- One short message a day at each person's agreed time (templates in section 6). Ask, do not lecture.
- Anyone under two sessions in a week gets a call: what got in the way, and one small change (shorter, earlier, a different voice).
- Log every contact in the participant's coaching log, the same day.
- Collect WHO-5 at the end of week 2.

THE PRODUCT TEAM

- A weekly 30-minute review with you: the top three blockers, ranked by how many people they hit.

GATE: PASSED WHEN

☐ 3+ sessions in both weeks 1 and 2: 70% or more

☐ WHO-5 week 2 answered: 85% or more

If it is not: Below 70% the study measures who had time, not whether Aurelia helps. Fix the blocker before week 3, even if it is not in the app.

See it in admin →

M3 Midpoint read, first iteration

Weeks 3–4

Read what is and is not working, and ship one change for the second half.

YOU

- Week 4: WHO-5, cohort measure, PHQ-9 and interview 2 with everyone.
- Bring the top three findings to the product team, each with the participants it affects and their own words.
- When the change ships, explain it to participants in one message: what changed and why.

THE PRODUCT TEAM

- Ships one change, as a new design version on staging first, by the start of week 5.
- Does not change the questionnaires or the timing: the measures must stay comparable.

GATE: PASSED WHEN

☐ Still enrolled: 85% or more

☐ Due questionnaires collected: 85% or more

☐ Iteration shipped to the group: confirmed by the product team

If it is not: Interim outcome figures are shown but not judged. A disappointing week 4 is information for the iteration, not a verdict.

See it in admin →

M4 Second half, second iteration

Weeks 5–7

Measure whether the change helped, and make one more adjustment if needed.

YOU

- Keep the daily rhythm. People who are winning need less; people who are stuck need a call a week.
- Collect WHO-5 at week 6.
- Follow up every safety alert the same day, and write down what you did.

THE PRODUCT TEAM

- Compares weeks 5–7 with weeks 1–4 for the people the change was for.
- A second change only if the first did not help, again on staging first.

GATE: PASSED WHEN

☐ Adherent people (3+ sessions in 60% of weeks): 60% or more

☐ Every safety alert followed up the same day: confirmed by the study lead

If it is not: If adherence falls in the second half, ask why in a call before changing anything else.

See it in admin →

M5

Final read and decision

Weeks 8–9

The pass lines decide: go, iterate or pivot.

YOU

- Week 8: WHO-5, cohort measure, PHQ-9, the value question and interview 3.
- Write one story per cohort: where the person started, what changed, in their words, with their consent.
- Present the result in week 9 against the pass lines set before week 0.

THE PRODUCT TEAM

- Prepares the decision: what scales, what changes, what stops.

GATE: PASSED WHEN

- ☐ Well-being improves: 50% gain 10+ WHO-5 points
- ☐ Their own issue improves: 40% reach the meaningful change
- ☐ People stay: 70% still enrolled
- ☐ Data to judge: 85% collected
- ☐ Would miss it: 40% "very disappointed"

If it is not: The lines were fixed before week 0. Moving them after reading the data turns the study into a story.

See it in admin →

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6. Your day and your week

A day

08:30

Open This week

Calls first, then anyone not reached for two days, then missed questionnaires.

Open →

09:00–11:00

Calls and follow-ups

Safety alerts the same day, always first. Then anyone stuck.

A week

Monday Milestones tab: which gate is next, and is it on track?

Wednesday Call everyone under two sessions last week.

Friday 30 minutes with the product team: the top three findings, with who they affect.

Every other week WHO-5 goes out; chase it within 48 hours.

The daily message, by issue

Each person's time

One message

Short, a question, about their own goal. Template below.

Same day

Log it

Coaching log on their page: what they said, how they feel 1–5, the next step.

Open →

16:30

Five-minute look

Wins and Engagement: who moved, who went quiet.

Open →

Bad sleep	*"Morning! How did last night go: roughly how long to fall asleep, and did you wake up?"*
Anxious	*"How is your day feeling, 1 to 5? Did the session before your meeting help, or not really?"*
Mental fog	*"Did you get a clear hour today? What did you play before it?"*
Low energy	*"How was your energy this afternoon compared with yesterday?"*
Distracted	*"How long did your longest focused stretch last today?"*
Stressed body	*"Where do you feel it today? Did the breathing chapter change anything?"*

One question, about their own goal, at the time they chose. Never “did you do your session?”.

When someone goes quiet

Missed a day

Your usual message; no comment on the miss.

Not reached for 2 days

They appear in This week. A second message, then a short call.

Under 2 sessions in a week

A call: what got in the way, and one small change.

Two questionnaires missed

A call, and offer to fill it in together.

Any sign of crisis or harm

Stop coaching. Crisis resources now, and the study lead the same day. Never handle it alone.

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7. Testing and iterating with the product team

The study is not only a measurement; it is how the product gets better while people use it. One loop, every week, and one significant change per half so its effect can be read.

1 · Hear

Coaching log, interviews, check-ins.

2 · Rank

How many people it hits × how much it stops them.

3 · Change

One change, as a new version on staging first.

4 · Explain

Tell participants what changed and why.

5 · Measure

Same measures, before and after the change.

Rules for a change

- At most one significant change in each half (M3, then M4 if needed).
- Staging first, as a new design version, then the group.
- The questionnaires and their timing never change.
- Every change is written down with the date it reached the group.

What a good finding looks like

“Four of six in the sleep cohort stop the session halfway because the voice is too fast (P02, P04, P05, P06; coaching log 3–5 Nov).” Who, how many, their words, where it is written.

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8. The admin is the source of truth

Everything you need to run and judge the study is in **admin → Experimentation**. If a question is not answered there, tell the product team; that is a gap in the tool, not something to keep in a spreadsheet.

YOUR QUESTION	WHERE	
Where are we, and is the next gate on track?	**Version 2 → Milestones**	Open →
What do I do today?	**Version 2 → This week**	Open →
Who has solved their issue?	**Version 2 → Wins**	Open →
Is use going with change?	**Version 2 → Engagement**	Open →
How is this one person doing?	**Participant page: measures, sessions, coaching log**	Open →
Are the pass lines on track?	**Version 1 → Overview**	Open →
Is any data missing?	**Version 1 → Data & safety**	Open →
What exactly do we collect, and when?	**Version 1 → What we collect**	Open →

Today the admin shows sample data, generated by the same rules the real data will use. The coaching log saves in your browser until the backend exists. Both are listed in the protocol as things to build before week 0.

9. Rules that never bend

Privacy

- Pseudonymous ids (P01–P30) everywhere; names stay outside every tool.
- No names or contact details in the coaching log.
- Journal and wearable data only with their own consent.

Safety

- PHQ-9 item 9 above zero, WHO-5 of 28 or below, or any report of harm: the study lead calls the same day.
- Crisis resources first, always.
- Aurelia is not a treatment and never replaces one.

Honesty

- Pass lines are fixed before week 0.
- People who leave stay in the count.
- No claim that Aurelia caused a change: there is no control group.

10. The decision at week 9

Go

Most pass lines met, including "their own issue improves" and "would miss it".

Plan a controlled trial and open a wider beta for the cohorts that won.

Iterate

People use it and stay, but outcomes are short of the lines.

A second eight-week cycle on the same design, with the changes the data points to.

Pivot

People do not use it even with daily guidance, or would not miss it.

Stop and rethink the core loop before more building.

11. Before week 0

READY?	WHAT	WHO
☐	Design, pass lines and this playbook approved	Product owner, Eugene
☐	Clinical advisor and ethics review; questionnaire licences (ISI, PSS-10)	Product owner
☐	Accounts that stay signed in; study enrolment and consent in the app	Product team
☐	Questionnaire screens with reminders; the safety alert to the study lead	Product team
☐	The admin on real data, including the coaching log	Product team
☐	30 people recruited, screened and consented; baseline calls booked	You

Appendix · The study protocol

The protocol of 10 November, in full: why the study, its design, the measures, the pass lines, how results are read, safety, and what must be built before week 0. The playbook above says how to run it; this says what it is. Sections are numbered P1 to P11.

People in the feedback group

30

DESIGN six cohorts of 4–6

Length of the study

8 weeks

DESIGN baseline, then weeks 1–8

Main question asked every two weeks

WHO-5

DESIGN well-being, 0–100

Pass line for well-being

≥ 50%

DESIGN improve by 10+ points

In one paragraph. So far we have tested whether people *understand and come back to* Aurelia: a simulation with six AI personas, then three real interviews. We have not tested whether it *helps*. This study does that in the simplest honest way. Thirty people with one target issue each (bad sleep, anxiety, mental fog, low energy, poor focus, a stressed body) use Aurelia for eight weeks. Every two weeks they answer a short, widely used well-being questionnaire (WHO-5), and at the start, middle and end a validated questionnaire for their own issue. The app records how often they actually use it. Before we start we fix the pass lines, so the result cannot be read to fit. The study can show whether people improve while using Aurelia and whether they keep using it, which is enough to decide whether the MVP deserves a proper controlled trial. It cannot prove that Aurelia caused the change, and we will not claim that it did.

P1. Why this study, and why now

Eugene's brief: a group of 30 people with different issues tries Aurelia for a period of time, to see whether they can "solve and win them": bad sleep to good sleep, mental fog to mental clarity. The admin needs a section to track and measure the group, as validation of therapeutic outcomes for the MVP.

WHAT WE HAVE	WHAT IT TOLD US	WHAT IT CANNOT TELL US
Retention simulation (25 Oct), six AI personas	Where first-time users get lost, and which fixes should bring them back	Anything about real people's outcomes; its numbers are predictions
Three real interviews (Ishmeet, Olivia, Bryan)	What feels untrustworthy, what they want (words, not scores; the why; their wearable)	Whether using Aurelia for weeks changes anything for them
The app (production v1 / v2)	A one-tap check-in after each session, a five-chapter journey, a journal	Validated outcome measures; the check-in is a mood tap, not a clinical measure

The study fills the gap between "people like it" and "it helps". It is also the first time real use is measured over weeks. That is why the work to keep people signed in (decision D2) becomes a must-have before week 0.

P2. The design at a glance

ELEMENT	CHOICE	WHY
Type	Single-arm, pre-registered pilot (everyone uses Aurelia; no control group)	The right size for an MVP. It tells us whether a controlled trial is worth running. A control group of 15 would be too small to show anything.
People	30 adults, recruited for one target issue each, never having used Aurelia	Matches the six goals onboarding already offers, so each person uses the product as it is
Length	8 weeks: baseline (week 0), then weeks 1–8	Long enough for sleep and stress to change; short enough to keep people. Eugene asked for two months.
What they do	Use Aurelia as they like, with a suggested rhythm of 3+ sessions a week and the five-chapter journey	We test the real product, not a lab version. Adherence is measured, not forced.
Main measure	WHO-5 Well-Being Index at weeks 0, 2, 4, 6, 8	Five questions, free to use, validated in many languages, with a known meaningful change (10 points)
Issue measure	One validated questionnaire per cohort at weeks 0, 4, 8	"Bad sleep to good sleep" needs a sleep measure, not only general well-being
Safety	PHQ-9 at screening, week 4 and week 8; same-day human follow-up on any risk answer	People with real difficulties will join; we must notice when someone is getting worse

ELEMENT	CHOICE	WHY
Identity	Pseudonymous ids (P01–P30) in every tool; names only with the research team	Health data. The admin never needs to know who someone is to measure the study.

P3. Six cohorts, one measure each

Each cohort is one of the goals people already choose in onboarding. Each has a validated questionnaire for its issue, and the change that counts as meaningful for one person.

COHORT	APP GOAL	MEASURE	RANGE	MEANINGFUL CHANGE	PLACES
Bad sleep → good sleep	Sleep better	Insomnia Severity Index (ISI)	0–28	6 points lower	6
Anxious → calm	Feel calmer	GAD-7	0–21	4 points lower	6
Mental fog → clarity	Clear my mind	PROMIS Cognitive Function 4a (T-score)	20–80	5 points higher	5
Low energy → energy	More energy	PROMIS Fatigue 4a (T-score)	30–80	5 points lower	5
Distracted → focused	Focus	PROMIS Cognitive Function 4a (T-score)	20–80	5 points higher	4
Stressed body → regulated	Regulate my body	Perceived Stress Scale (PSS-10)	0–40	4 points lower	4

The meaningful-change figures are the thresholds commonly used in the research literature for these instruments. A clinical advisor should confirm them before week 0, together with licensing: WHO-5, PHQ-9, GAD-7 and PROMIS are free to use; ISI and PSS-10 may need permission for commercial use. Sleep and calm are the largest cohorts because they had the largest shares in the simulated panel. That is an assumption about early users, not a measurement.

P4. The eight weeks

Week 0	Week 1	Week 2	Week 3	Week 4	Week 5	Week 6	Week 7	Week 8
Consent, screening (PHQ-9),	Use Aurelia; check-ins;	WHO-5	Use; weekly one-liner	WHO-5, cohort measure,	Use	WHO-5	Use	WHO-5, cohort measure,

WHO-5, cohort measure, interview 1, onboarding	sleep diary for the sleep cohort			PHQ-9, interview 2 · interim read				PHQ-9, value question, interview 3 · final read

Every session: a before-play tap and the existing "How do you feel now?" check-in. Daily, if the person agrees: wearable sleep, resting heart rate and HRV. Any time: "Something felt wrong", reviewed by a person.

P5. The data we need to collect

This is the list Eugene asked for. The last column says whether production can collect it today. Most of it has to be built before week 0 (section P10).

MEASURE	WHY WE NEED IT	WHEN	IN THE APP TODAY
WHO-5 well-being, 0–100	The one comparable outcome across all six issues; the primary result	Wk 0, 2, 4, 6, 8	NO
Cohort measure (table above)	Whether their own issue improved: "bad sleep to good sleep"	Wk 0, 4, 8	NO
PHQ-9 safety screen	To notice anyone getting worse; item 9 triggers a same-day call	Screening, wk 4, 8	NO
After-session check-in 1–5	Short-term effect of each session	Every session	YES needs saving to the account
Before-session state 1–5	Turns the check-in into a before/after for each session	Every session	NO
Sleep diary (sleep cohort)	Time to fall asleep, night wakings: the details ISI summarises	Every morning	NO
Wearable sleep, resting heart rate, HRV	An objective signal next to what people report; all three interviewees wear one	Daily, if consented	PARTLY v2 asks; connection needs backend
Engagement: sessions played, minutes, chapters, journal	Without use there is no effect to measure; it also separates "did not help" from "did not use"	Continuous	PARTLY views and taps only
Weekly one-liner and **interviews**	The why behind the numbers	Weekly; wk 0, 4, 8	NO research team

MEASURE	WHY WE NEED IT	WHEN	IN THE APP TODAY
Value question (Sean Ellis) and would-recommend	Would they miss it: the product-market-fit signal	Wk 8	NO
Adverse events	Anything that felt worse, at any time	Any time	NO

P6. What counts as success

Set before week 0 and not changed afterwards. Outcome lines are judged only at week 8; the admin shows interim figures as "not yet".

QUESTION	PASS LINE	WHY THIS LINE
Does well-being improve meaningfully?	≥ 50% of people gain 10+ WHO-5 points by week 8	10 points is the change the WHO-5's authors call meaningful; half the group is a strong signal for a pilot
Does their own issue improve?	≥ 40% reach the meaningful change on their cohort measure	Issue measures move more slowly than general well-being
Do people stay?	≥ 70% still enrolled at week 8	Below that, the people who remain are not representative
Do people actually use it?	≥ 60% play 3+ sessions in at least 60% of weeks	An effect without use would be noise; use without effect is its own finding
Do we have the data to judge?	≥ 85% of due questionnaires answered	Missing data weakens every other line
Is anyone harmed?	No serious adverse event; every alert followed up the same day	Non-negotiable in a mental-health-adjacent product
Would they miss it?	≥ 40% "very disappointed" (Sean Ellis)	The usual product-market-fit bar, also used in the retention test plan

P7. How we will read the results

- **Responder rates with their uncertainty.** With 30 people, a 50% responder rate has a 95% range of roughly 32–68%, so we report the range, not just the number.

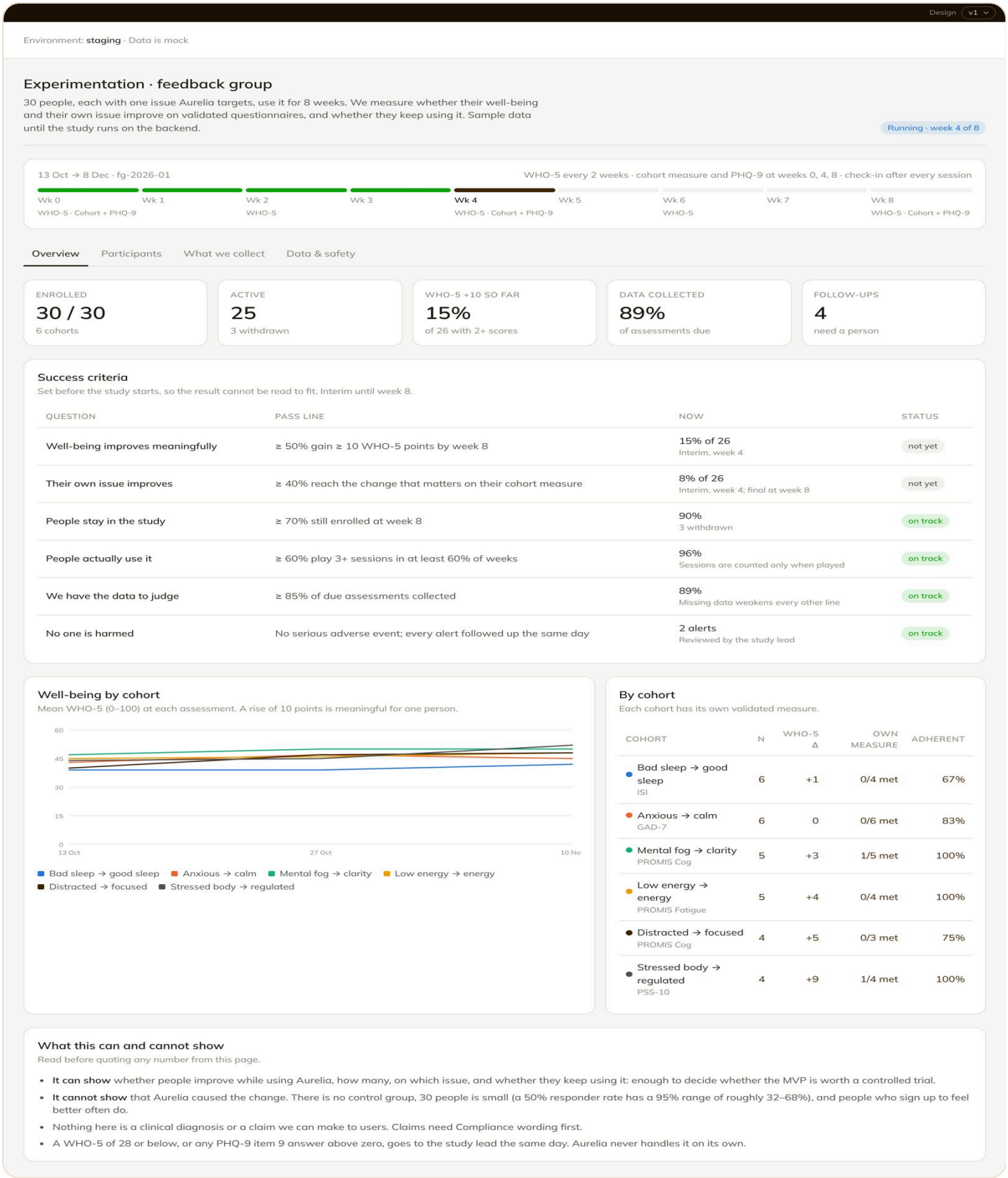
- **Change from baseline** on WHO-5 and each cohort measure: the average change, a paired test (Wilcoxon signed-rank), and an effect size.
- **Cohorts are described, not tested.** With 4–6 people each, a cohort result is a direction, not a finding.
- **Everyone counts:** people who withdraw stay in the denominator ("intention to treat"). We also report the result for people who used it as suggested.
- **Use is related to change:** do people who played more improve more? This is the closest a single-arm study gets to a dose signal.
- **What we will not claim:** that Aurelia caused the change, that it treats any condition, or any number in marketing. People who join a study to feel better often do; regression to the mean and expectation both push scores up. A controlled trial is the next step if the pass lines are met.

P8. Safety, consent and privacy

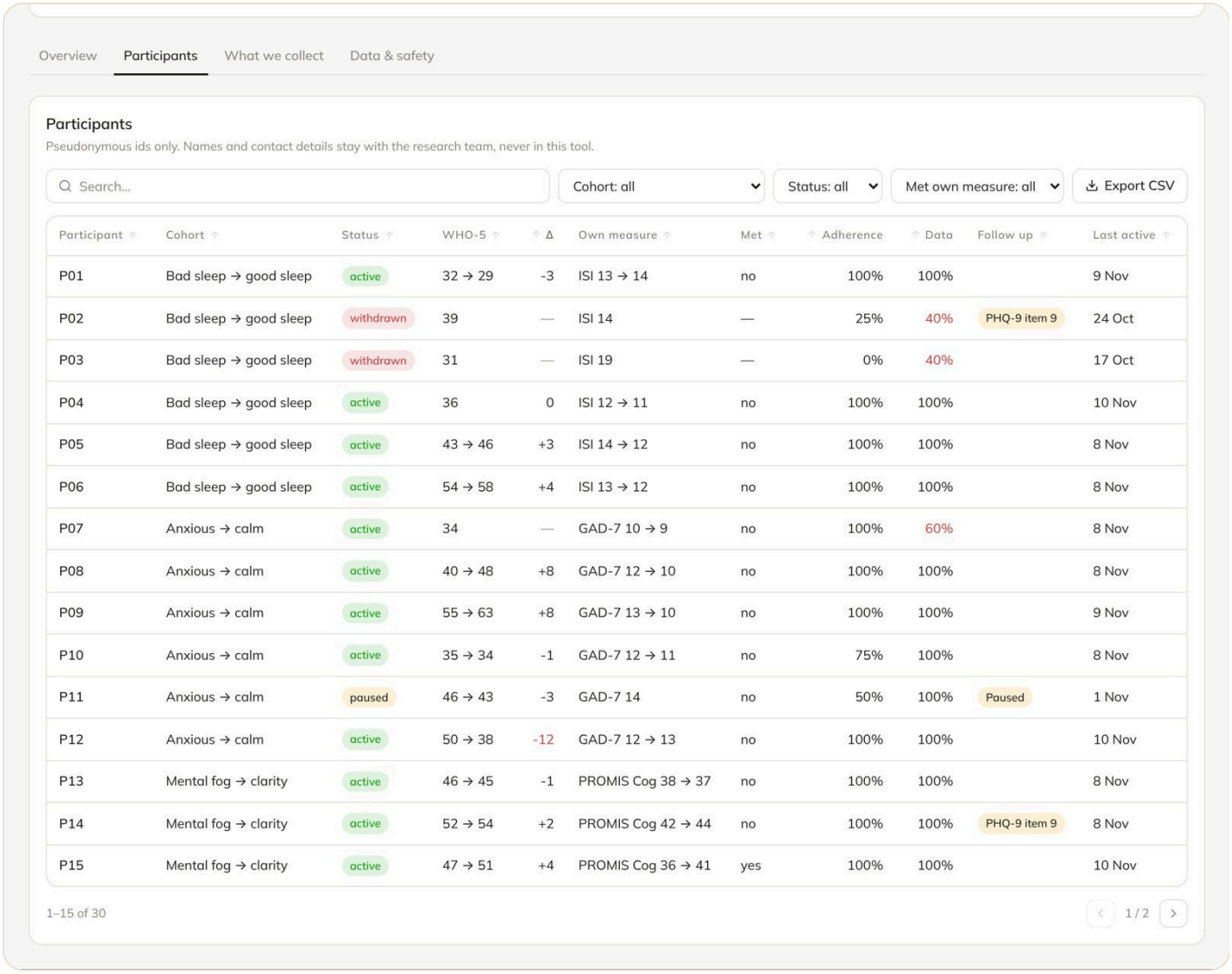
- **Informed consent** before any data is stored: what is collected, who sees it, the right to withdraw at any time, and that Aurelia is not a treatment. Anyone in treatment keeps it; Aurelia is never a replacement.
- **Who should not join:** people in active crisis, with recent psychiatric hospitalisation, or scoring 20+ on PHQ-9 at screening. They are offered support resources instead.
- **Risk answers go to a person:** a PHQ-9 item 9 above zero, a WHO-5 of 28 or below, or a reported adverse event goes to the study lead, who calls the same day. Aurelia never handles it on its own, and the participant never sees a PHQ-9 score.
- **Privacy:** pseudonymous ids in every tool, journal text never leaves the participant's account without consent, and wearable data only with a separate consent.
- **Review:** an independent ethics review and a clinical advisor before week 0 are strongly recommended. They are needed if the results are ever to be published or used with partners.

P9. The Experimentation section in admin

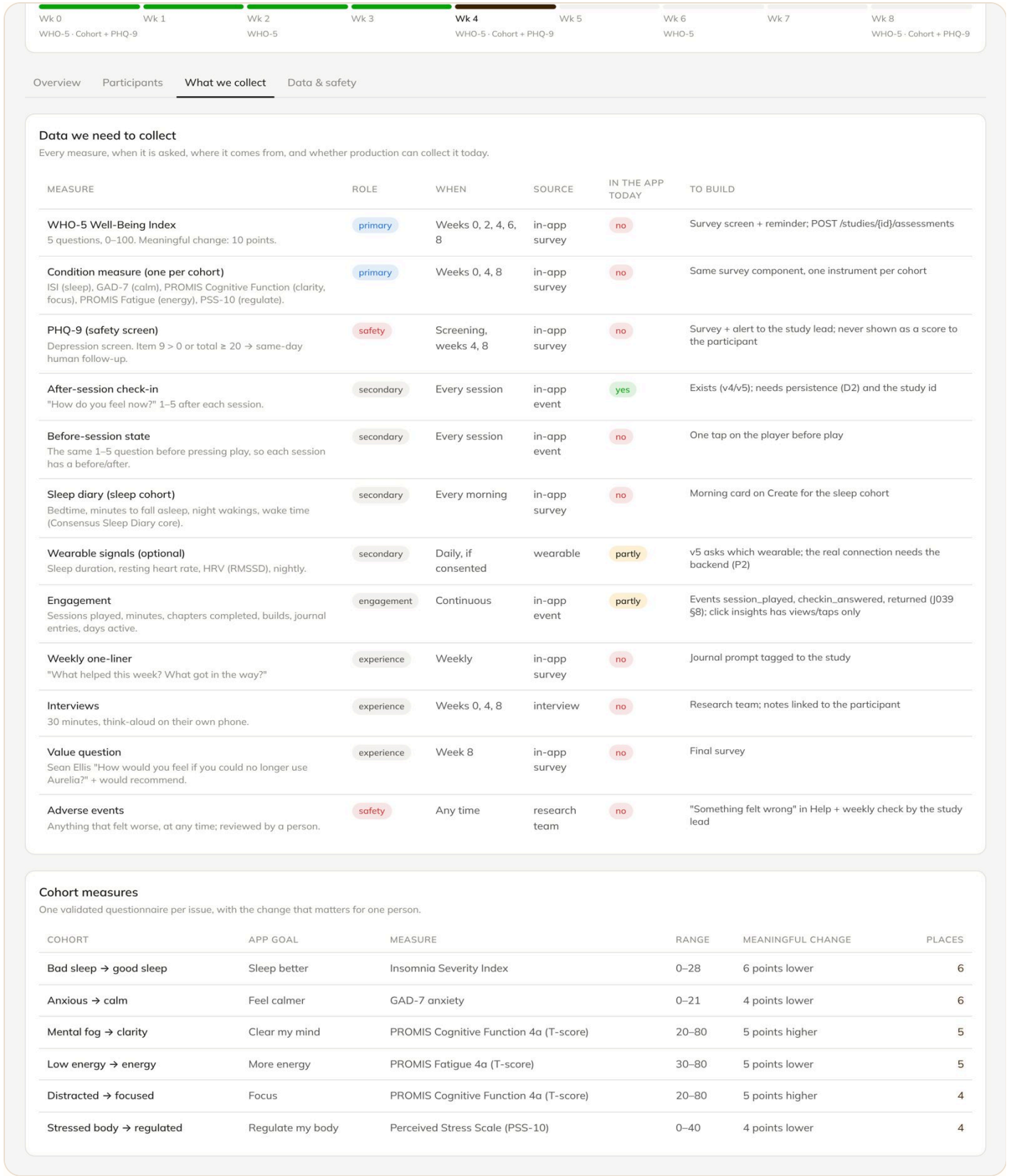
In admin: **Experimentation → Feedback group · v1** (these four views and a page per participant). Since 11 November there is also **· v2** (Milestones, Wins, This week, Engagement) and a coaching log on each participant page; see section 8. Until the study runs on the backend it shows sample data, generated by the same rules the real data will use. The screenshots below are mid-study (week 4 of 8) on sample data.



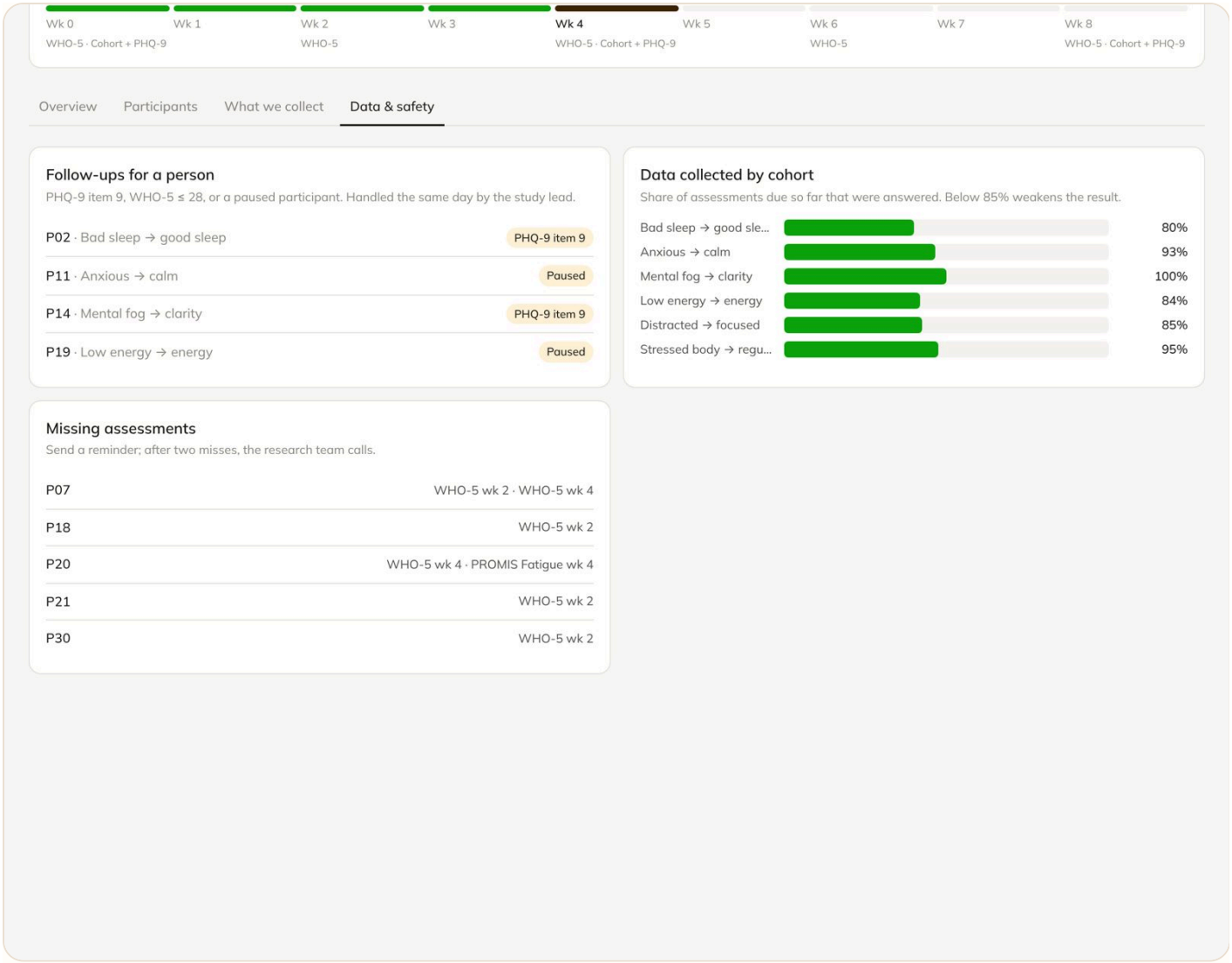
Overview. A timeline of the eight weeks and what is collected when; headline figures; the success criteria with their status (outcome lines "not yet" until week 8); well-being by cohort; a cohort table; and a box on what the study can and cannot show.



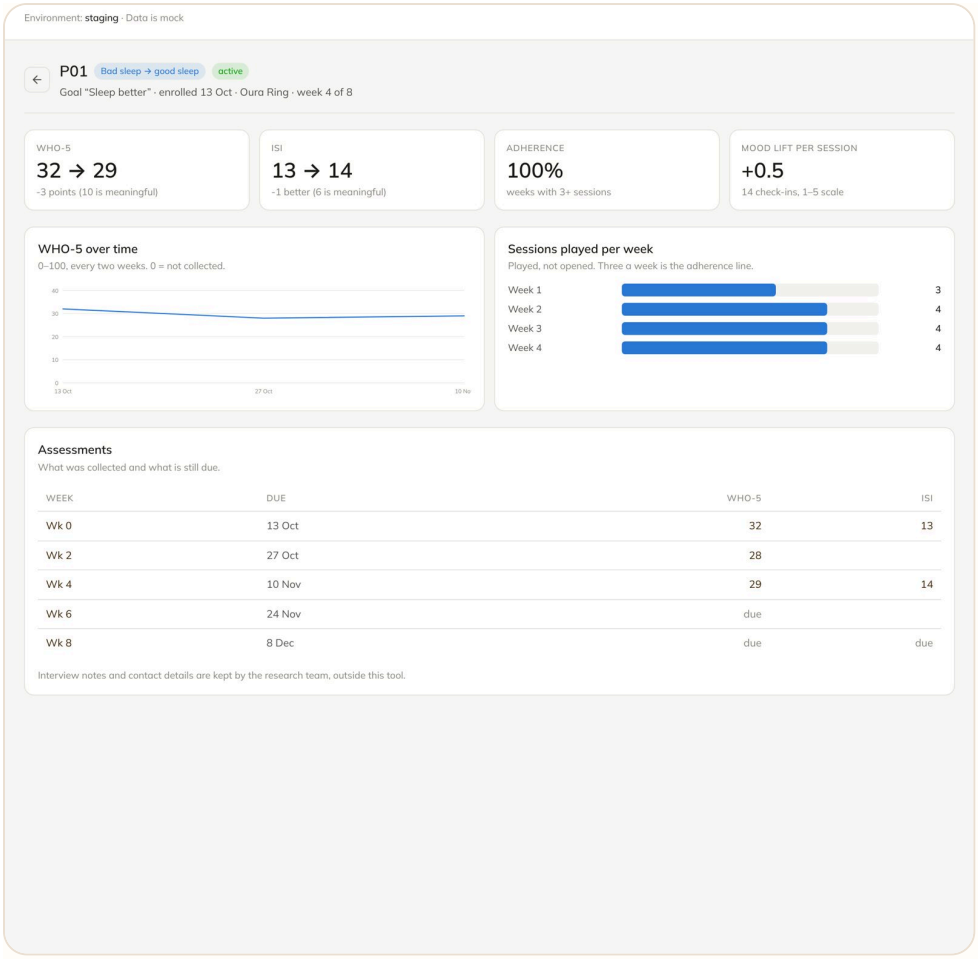
Participants. Thirty pseudonymous rows: cohort, status, WHO-5 before and now, their own measure, adherence, data completeness and follow-up flags. Filter by cohort, status or whether they met their measure; export to CSV.



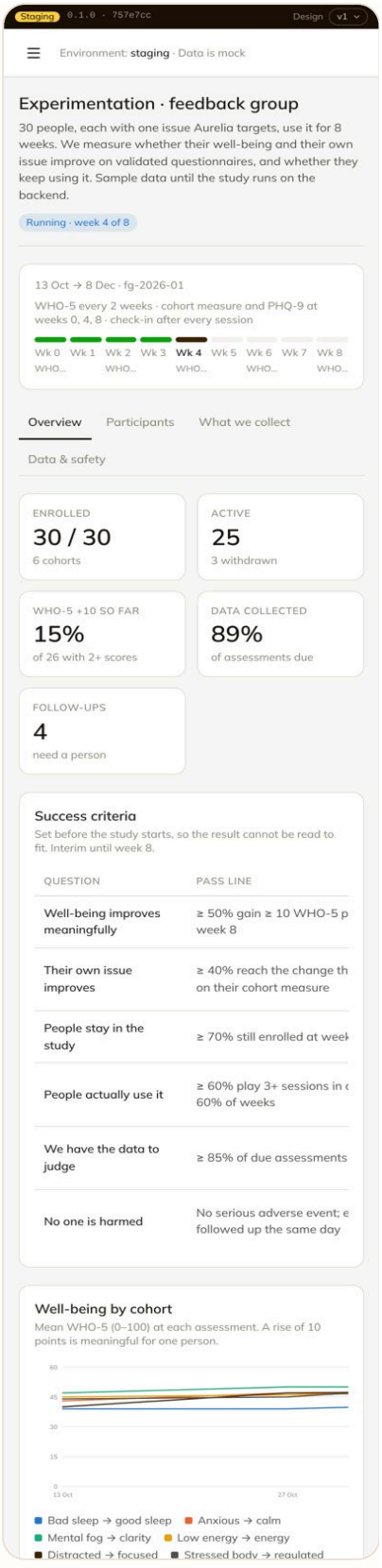
What we collect. The protocol from section P5 inside the tool, with what each item needs to be built, plus the cohort measures.



Data & safety. Follow-ups for a person, data collected per cohort, and every missing questionnaire, so nobody silently drops out of the data.



One participant. WHO-5 over time, their own measure, adherence, mood lift per session, sessions per week, and each questionnaire: done, missed or due. Contact details stay with the research team.



The same overview on a phone.

P10. What must be built before week 0

ITEM	WHY	STATUS
Stay signed in, with data saved to the account (D2)	Eight weeks of check-ins cannot live in one browser	NEEDS BACKEND
Study enrolment and consent in the app	Links a person to fg-2026-01, a cohort and a pseudonymous id	TO BUILD
Questionnaire screen (WHO-5, cohort measures, PHQ-9) with reminders	The primary data; the API contract now includes it (§5.12)	TO BUILD
Before-play tap, sleep diary card, weekly one-liner	Secondary measures	TO BUILD
Events: session played, check-in answered, returned	Engagement and adherence	TO BUILD
Safety alert to the study lead	Same-day follow-up	TO BUILD
Wearable connection (Apple Health first)	Optional objective signal	OPTIONAL FOR WEEK 0
Admin Experimentation section	Tracking and measuring the group	ON STAGING (sample data)

If the backend is not ready, the study can start with the questionnaires in a form tool and the admin fed by a weekly import. That is slower and leaves more gaps, so it is a fallback, not the plan.

P11. Decisions needed from you

1. Approve the design and pass lines

They must be fixed before the first person starts, or the result can be read to fit.

2. Who recruits, and with what incentive?

Recommended: two people per issue from outside the team, with free Pro for a year after completing.

3. A clinical advisor and an ethics review

To confirm the measures, the thresholds and the safety process.

4. Start date

Week 0 starts when the items in section P10 are ready; about three to four weeks of build after a yes.

The appendix is the study protocol (`docs/reviews/Aurelia-Feedback-Group-Study-2026-11-10.pdf`), read from its HTML. The milestones are computed in admin by `src/admin/data/study.ts` ; this document is generated by `scripts/guides/pm_playbook.py` . AI analysis by Claude, based on its own analysis of the app and deep research.